NCCP 00585 · clinician draft

Fortnightly cetuximab + FOLFIRI

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Irinotecan + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Antihistamine	As prescribed in the local cetuximab premedication plan	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg PO once, subject to reconciliation with the cetuximab premedication prescription	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan when prescribed; assess acute cholinergic symptoms Avoid in glaucoma. This treats/prevents the acute cholinergic syndrome; it is not the delayed-diarrhoea rescue medicine. Use caution in asthma, as specified in the NCCP acute cholinergic-syndrome advice.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic G-CSF schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Provide a written irinotecan delayed-diarrhoea rescue and escalation plan before treatment; distinguish it from immediate infusion-related cramps/sweating. Fever, severe pain, dehydration or diarrhoea not responding to the prescribed plan needs urgent review.
- Confirm line care, pump contact details and disconnection arrangements; report chest pain, severe early fluorouracil toxicity or pump/line problems urgently.
- Supply a written irinotecan delayed-diarrhoea rescue prescription: NCCP reference loperamide 4 mg initially, then 2 mg every 2 hours; continue for 12 hours after the last liquid stool, never longer than 48 hours at this high-dose schedule. Start advised electrolyte-containing fluids at the first liquid stool and contact the team early. Fever, dehydration, severe pain or persistent symptoms require urgent oncology review.
- Irinotecan treatment-related diarrhoea must fully resolve before restart; a table-grade threshold alone does not clear the next dose.
- If treatment is delayed for cytopenia, follow the source-specific weekly blood-count checks and maximum delay; assess the antibody and chemotherapy components separately when indicated.
- EMA Erbitux section4.2: chemotherapy must not begin earlier than one hour after the end of cetuximab.
- Observation attribution: NCCP permits stopping routine post-infusion observation after two tolerated doses; EMA Erbitux specifies close monitoring during and for at least one hour after infusion. Reconcile the approved local observation policy; do not automate removal of monitoring.
- Medication review: NCCP contraindicates concomitant brivudine, sorivudine and analogues with fluorouracil; review DPD-affecting medicines, phenytoin toxicity/levels and warfarin INR.
- Review CYP3A inducers/inhibitors and grapefruit; avoid prochlorperazine on the same day as irinotecan. Gilbert syndrome warrants consideration of a reduced starting irinotecan dose; no universal percentage is supplied.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00585 v5b · p6 · NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required